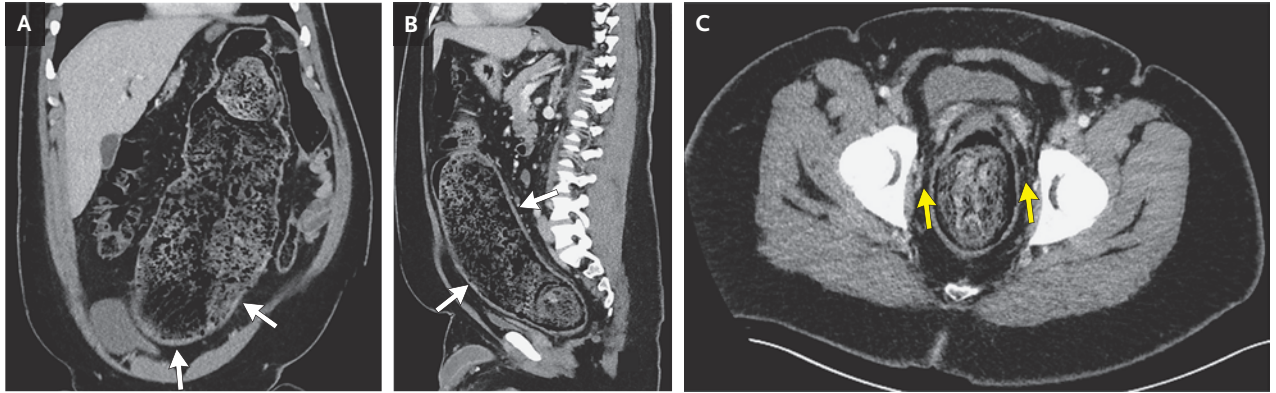


IMAGES IN CLINICAL MEDICINE

Stercoral Colitis



A 23-YEAR-OLD MAN WITH AUTISM SPECTRUM DISORDER AND RECURRENT hospitalizations for severe constipation presented to the emergency department with a 1-week history of abdominal pain on his left side, nausea, and vomiting. Owing to behavioral challenges, the patient had difficulty adhering to a bowel regimen for chronic constipation as an outpatient. A previous evaluation for Hirschsprung's disease had been negative. Physical examination was notable for abdominal distention and mild tenderness to palpation on the left side of the abdomen. The serum lactate level was normal. Computed tomography of the abdomen and pelvis revealed distention of the colon by a large stool burden and mural thickening (Panels A and B, white arrows) and perirectal fat stranding (Panel C, yellow arrows). (Panels A, B, and C show the coronal, sagittal, and axial views, respectively.) A diagnosis of stercoral colitis was made. In stercoral colitis, chronically impacted feces distend the colon, resulting in inflammation. In some cases, the fecaloma may lead to focal-pressure necrosis or perforation. Treatment with an aggressive bowel regimen was given, and fecal disimpaction was performed by means of flexible sigmoidoscopy. At discharge, the patient was having several bowel movements per day on a bowel regimen. At a follow-up visit at the gastroenterology clinic approximately 5 months after discharge, anorectal manometry showed non-relaxation of the anorectal angle, which was thought to be related to puborectalis muscular dysfunction. Pelvic-floor physical therapy was initiated.

Aleksandra Bajer, B.S.¹Erica Levine, M.D.¹¹University of Minnesota, Minneapolis.Erica Levine can be contacted at elevine@umn.edu.

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